NCCP 00776 · VERSION 3B

Trastuzumab deruxtecan (Enhertu) · every 3 weeks

One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Nausea and vomiting, including delayed symptoms

☐ Fatigue, reduced appetite and hair loss

☐ Low blood counts: infection, anaemia or bleeding

☐ Interstitial lung disease / pneumonitis, sometimes fatal

☐ Reduced heart pumping function or heart failure

☐ Infusion reactions; liver-test abnormalities

☐ Harm to an unborn baby and possible fertility effects

Enhertu is different from trastuzumab and trastuzumab emtansine (Kadcyla); they are not interchangeable. Carry the supplied Enhertu lung-safety alert card. The team may pause treatment while investigating suspected lung inflammation; symptomatic drug-related ILD requires permanent discontinuation under the protocol. Where relevant, use effective contraception during treatment and for at least seven months after the last dose for women, or four months for men with a partner who could become pregnant. Do not breastfeed during treatment and for seven months after the last dose. Discuss fertility before starting.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before the first dose: full blood count, kidney and liver tests, an echocardiogram or MUGA scan to measure heart pumping function, and a pregnancy test where relevant. The team checks the HER2 result, previous treatment, lung history and current breathing symptoms, and agrees how disease response will be assessed. Blood count, kidney and liver tests before each 21-day cycle. Review breathing, fatigue, nausea, intake and other symptoms at each visit. Repeat cardiac imaging every 12 weeks; assess new breathing symptoms immediately rather than waiting for the next planned scan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.